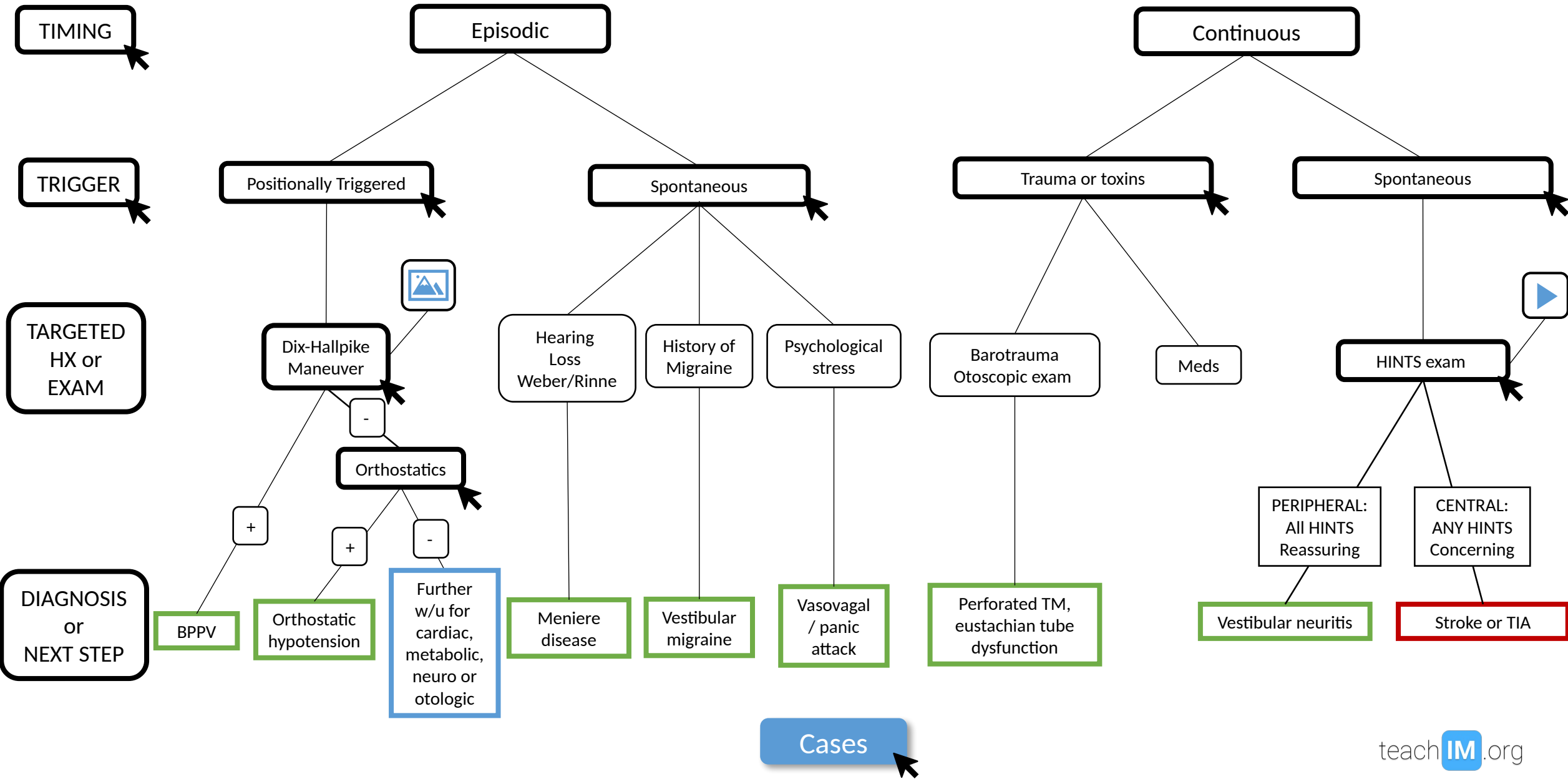


Approach to Acute Dizziness



Case 1

60 yo woman, former smoker with HTN and CAD presents with dizziness. The episodes started two days ago, triggered by turning her head and last less than a minute. She has some nausea with the episodes but no vomiting. It is interfering with her ability to do her daily activities. No change in hearing, tinnitus, HA, vision changes, weakness, falls.

Meds: Atorvastatin 80 mg, Lisinopril 20 mg

How do you categorize her symptoms?

Episodic, positionally triggered

What exam or history will you focus on?

Dix-Hallpike. [Here](#) is her exam

What is the diagnosis

BPPV

What management would you offer?

[Epley Maneuver](#) – PT referral
[Half- Somersault](#) – at home

Case 2

60 yo woman, former smoker with HTN, CAD, DM presents to the ED with sudden onset of persistent vertigo, stating about 1 hour ago. She has a history of BPPV but was not able to perform Epley maneuver at home due to the severity of her dizziness. She is unable to do anything. No change in hearing, tinnitus, vision changes.

Meds: Atorvastatin 80 mg, Lisinopril 20 mg

How do you categorize her symptoms?

Continuous, spontaneous

What exam or history will you focus on?

HINTS exam. [Here](#) is her exam

What is the diagnosis

Acute CVA

What management would you offer?

Non-con HCT, MRI/MRA, antiplatelet

Case 3



60 yo woman, former smoker with HTN and CAD presents with three days of a continuous sensation of the room spinning. She has had been lying in bed unable to work, with nausea but not vomiting. She attempted the half-somersault and Epley maneuvers at home, but they did not improve her symptoms. No change in hearing, tinnitus, HA, vision changes, weakness, falls.

Meds: Atorvastatin 80 mg, Lisinopril 20 mg

How do you categorize her symptoms?

Continuous, spontaneous

What exam or history will you focus on?

HINTS exam. [Here](#) is her exam

What is the diagnosis

Vestibular neuritis

What management would you offer?

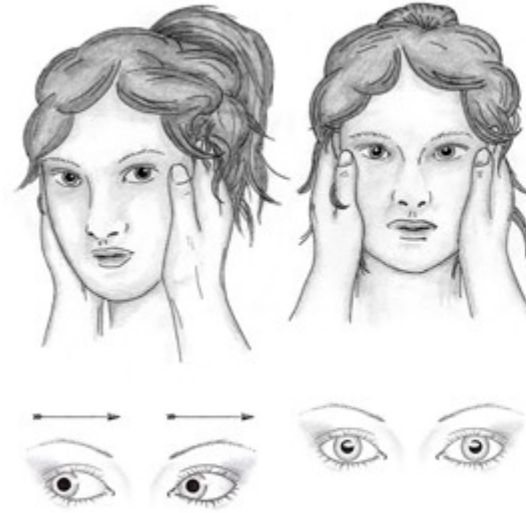
Anti-emetics & anticholinergics
? Steroids ?



Head Impulse



NO catch-up saccades = concerning
(brain problem)



Nystagmus



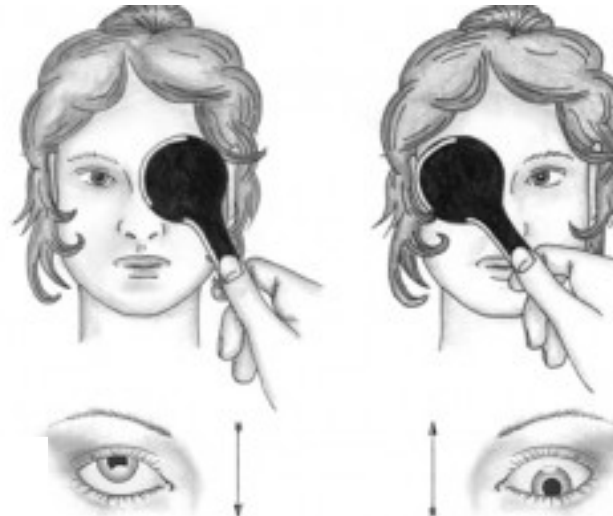
Bidirectional = Concerning



Test of Skew



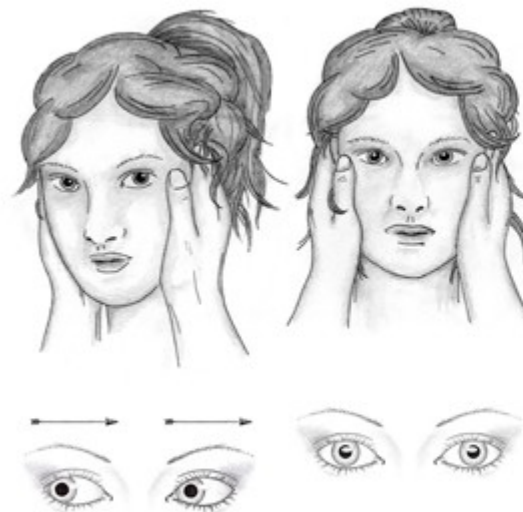
+ Vertical deviation = Concerning





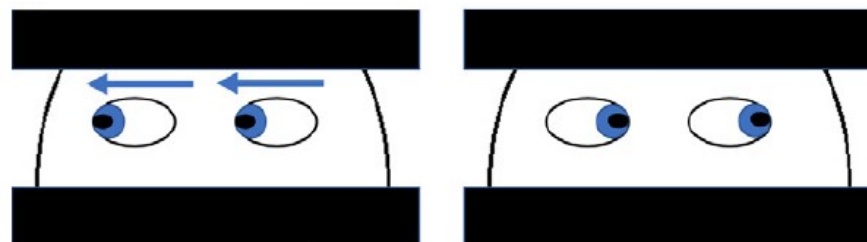
Head Impulse

Catch-up saccades = reassuring
(nerve problem)



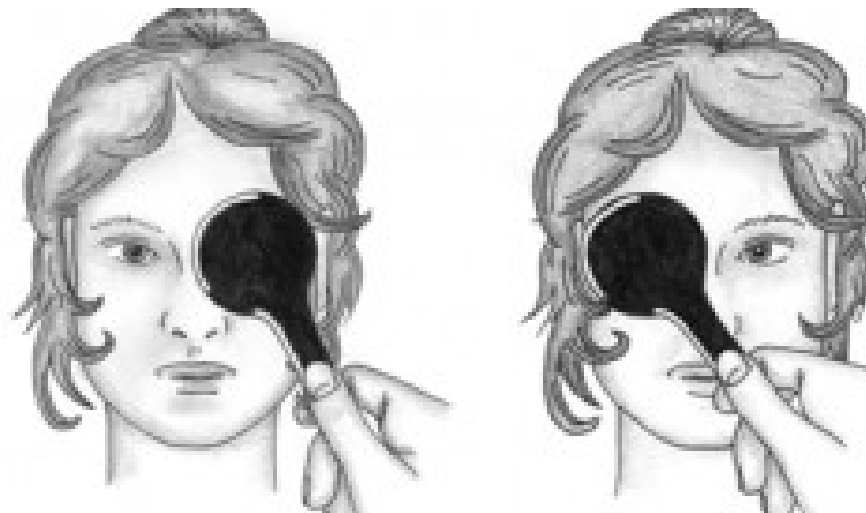
Nystagmus

Unidirectional = reassuring



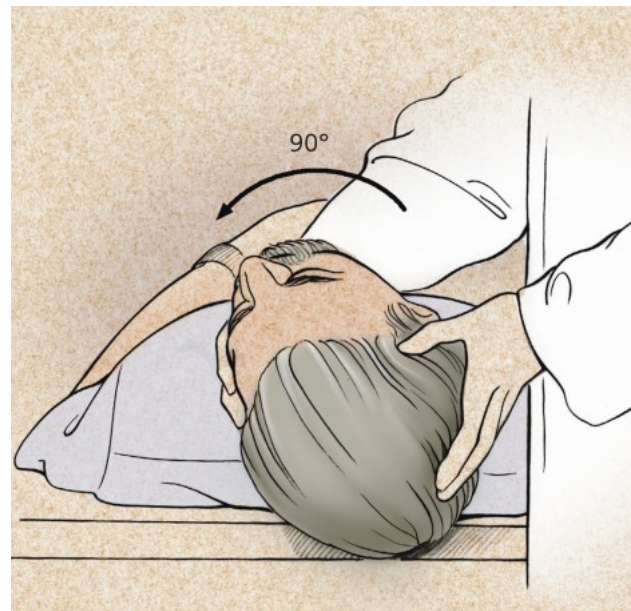
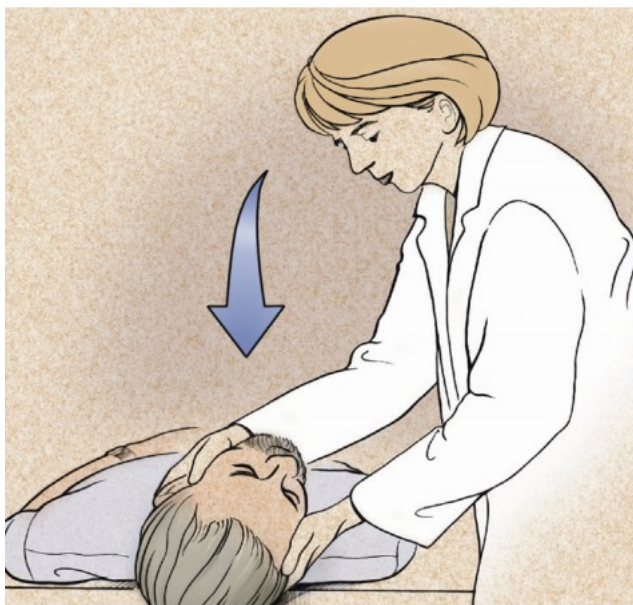
Test of Skew

No vertical deviation = reassuring



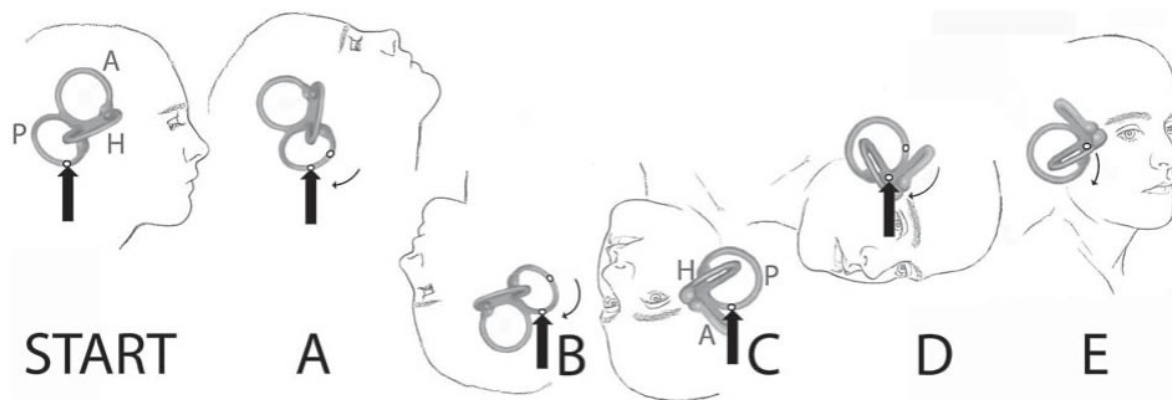
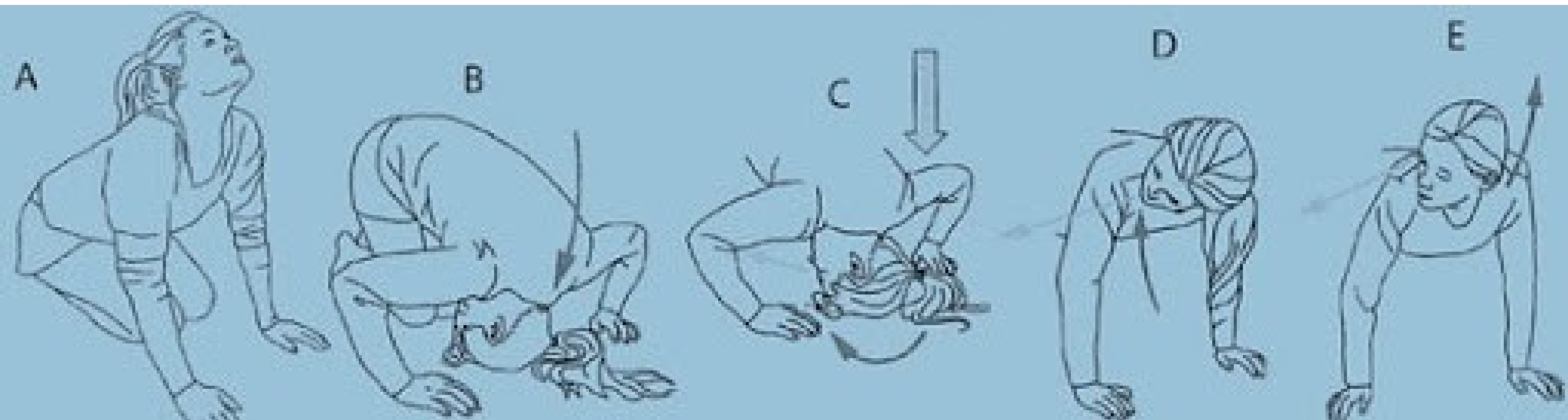


Epley maneuver



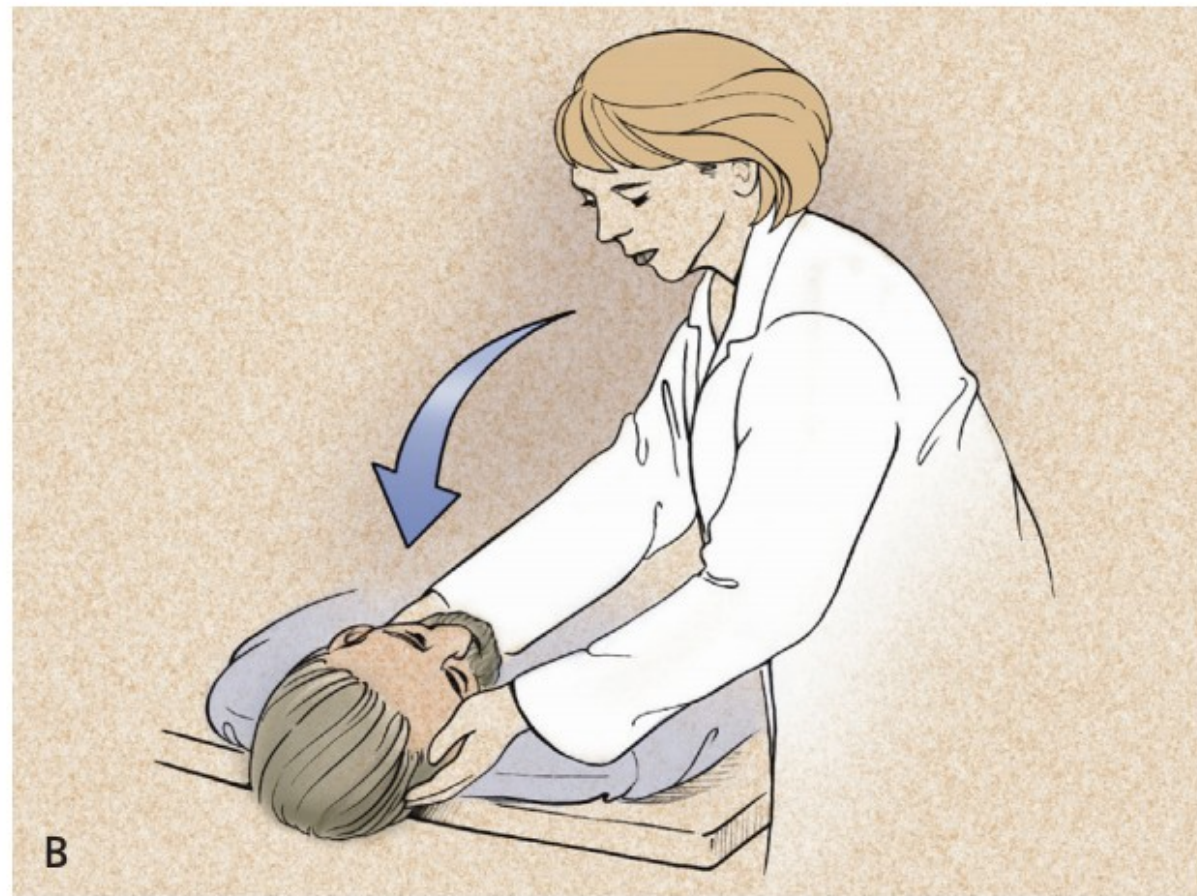
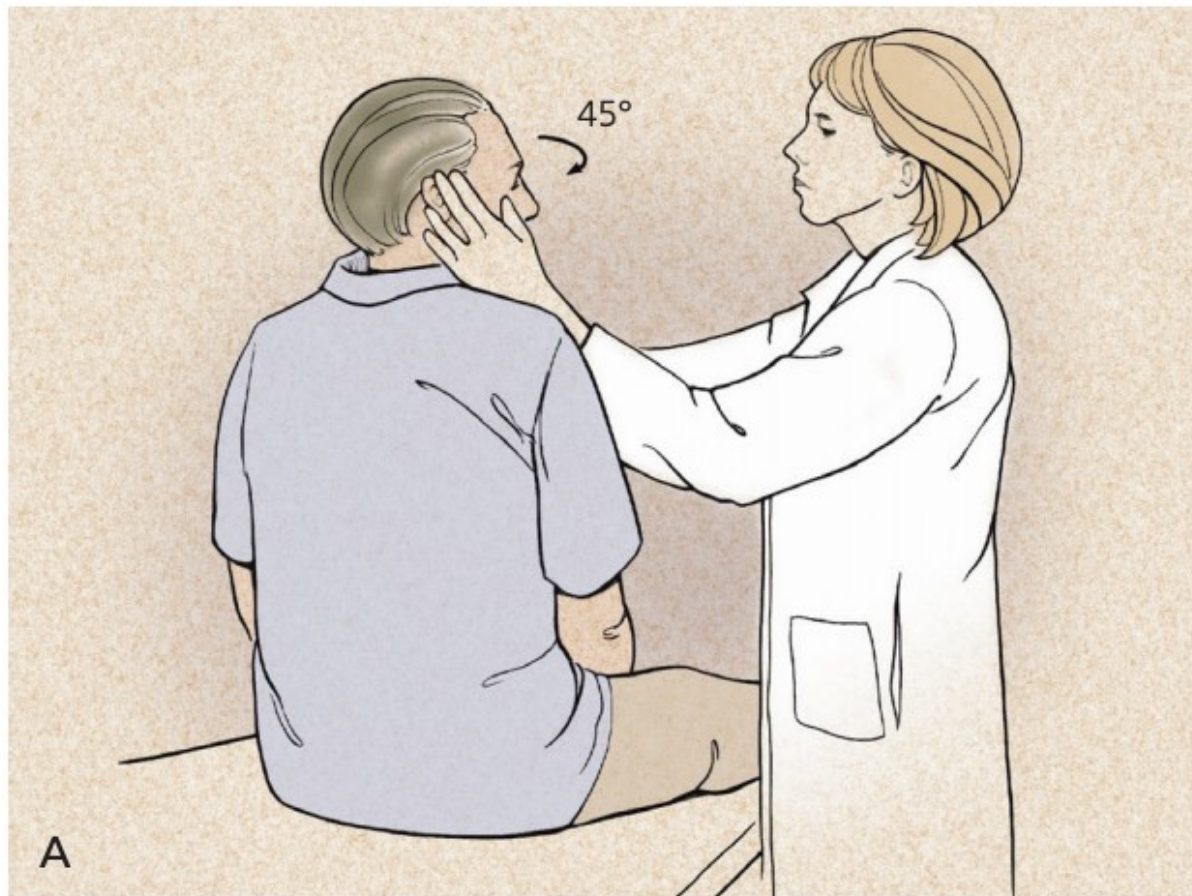


Half-somersault maneuver





Dix-Hallpike maneuver



ILLUSTRATIONS BY MARCIA HARTSOCK

2023 Update — GRACE-3 (SAEM)

Standing teaching (TiTrATE timing/triggers, Dix-Hallpike, HINTS) remains valid; GRACE-3 (Edlow, Acad Emerg Med 2023) refines ED practice:

HINTS only in ACUTE VESTIBULAR SYNDROME with nystagmus, by TRAINED examiners — inaccurate when applied to wrong patients or by untrained clinicians; do not use in triggered/episodic dizziness.

Strong rec AGAINST routine noncontrast head CT to distinguish central vs peripheral (poor sensitivity for posterior-fossa ischemia).

pc-BPPV with +Dix-Hallpike: perform Epley canalith-repositioning at time of dx (strong rec).

Vestibular neuritis: short-course steroids only via shared decision-making if within 3 days — weak/very-low-certainty evidence.

STANDING (timing+triggers framework) Newman-Toker, Neurol Clin 2015; HINTS Kattah, Stroke 2009 — unchanged.

Take-Home Points

Use Timing + Triggers (modified TiTrATE) to sort acute dizziness into 4 categories before choosing a bedside test.

Episodic + positionally triggered → Dix-Hallpike; if positive = pc-BPPV, treat with Epley at the bedside.

Continuous + spontaneous (acute vestibular syndrome) → HINTS; ANY concerning finding (no catch-up saccade, bidirectional/vertical nystagmus, skew) = central until proven otherwise.

HINTS is reliable only in continuous dizziness with nystagmus, performed by a trained examiner — and out-beats early MRI-DWI for posterior stroke.

Don't rely on noncontrast CT to exclude posterior-circulation stroke.

Attribution & Changelog

Adapted from TeachIM 'Acute Dizziness' (Meara Melton MD; ed. Brandon Fainstad MD), pub. March 2021.

teachim.org/teaching_material/acute-dizziness/

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Surgical correction: 'Dix-Halpike' -> 'Dix-Hallpike' (slides 1 & 2; slide 7 already correct).

Added (source-cited): 2023 GRACE-3 ED dizziness guideline context (HINTS use limits, CT rec-against, Epley at bedside, steroid shared decision-making).

Appended: this update slide, take-home slide, attribution slide. All original animated/click-to-reveal builds preserved.

Optimization date: 2026-06-10. Educational use only; clinician-review medical content before teaching.